East of England Cancer Alliances

NHS

PROSTATE SUSPECTED CANCER REFERRAL FORM

Date of GP decision to refer: No. of pages sent:

NOTE: This form is NOT for use for patients under 16

INFORMATION PROVIDED TO PATIENT (To be provided by referring Clinician)	Please tick
Patient has been informed that cancer needs to be excluded	☐
Patient has been given written information leaflet regarding the Urgent Suspected Cancer pathway	☐
Patient understands that they may go straight to test	☐
Patient has confirmed they are available for the next 7 to 14 days	☐
Patient has confirmed they can accept earliest appointment and may be offered an appointment at any of the hospital sites	☐

PATIENT DETAILS – **Must** provide current telephone number				
Last name:		First name:		
Gender: Sex at birth *(Male, Female, Intersex*		DOB:		
NHS No:				
Address:				
Tele (Day/Work):		Mobile No:		
Email:		Patient happy for a message to be left		☐
Language:				

GP DETAILS	
GP name:	
Practice Code:	
Address:	
Telephone:	
Practice email: ***(ESSENTIAL** – to improve communication)*	

REASON FOR REFERRAL	
☐	**PROSTATE:** Feels malignant on digital examination *(if performed).* If PSA is normal use A&G: *prostate cancer very unlikely in this scenario*
☐	**PROSTATE:** PSA levels above age-specific threshold *(considering patients preference and comorbidities)*
☐	**PROSTATE:** Symptoms suggestive of metastatic prostate cancer *(bone pain, unintended weight loss and fatigue)* if their PSA levels are above age-specific threshold

If unsure USC criteria are fulfilled, please use A&G

CLINICAL INFORMATION (Include any Radiology imaging if relevant)

ESSENTIAL PRE-REFERRAL INVESTIGATIONS

Prostate-Specific Antigen (PSA) Threshold ranges:

- Below 40 years Use Clinical judgement
- 40-49 years 0 – 2.5ng/L
- 50-59 years 0 – 3.5ng/L
- 60-69 years 0 – 4.5ng/L
- 70-79 years 0 – 6.5ng/L
- Above 79 More than 20 or more than 7.5 and there are symptoms suggestive of metastatic disease (bone pain and/or fatigue and/or significant unintended weight loss)

Data from: [NICE, 2021c; AoMRC, 2024]
Note: NICE reports micrograms/L, which is equivalent to nanograms/mL used by AoMRC.

Before a PSA test, people should not have:

- An active urinary infection or within previous 6 weeks.
- Ejaculated in previous 48 hours.
- Exercised vigorously, for example cycling, in the previous 48 hours.
- Had a urological intervention such as prostate biopsy in previous 6 weeks.
- In those men above the age of 79
  - If the initial PSA test is 7.5 - 20ng/mL and there are no symptoms suggestive of metastatic disease repeat the PSA once after 6 months.
  - If PSA has doubled after 6 months then refer.
  - *Please see NICE CKS for full guidance*

Investigations		***Results:***	***Dates:***
PSA *(4 weeks apart)*	1st Test		
	2nd Test		
Creatinine	(within last 3 months to *support STT pathways)*		
eGFR *(Include if creatine not in normal range)*	(within last 3 months to *support STT pathways)*		
UTI excluded	*Include Date treated*		
MSU Result			
Other, please give details:			

ROCKWOOD PERFORMANCE SCORE

Accurate functional status is needed to assess the most appropriate investigation/s and treatment.

If not known please select a score from the attached link for the Rockwood scale and put in the score box:
https://www.england.nhs.uk/south/wp-content/uploads/sites/6/2022/02/rockwood-frailty-scale_.pdf

SCORE:

The Rockwood Clinical Frailty Scale has not been widely validated in younger populations (below 65 years of age). If either apply please provide details of your patient's support requirements in the Additional Considerations section.

ADDITIONAL CONSIDERATIONS

Transport required?	☐	**If yes, please provide further details:**
Language / Hearing / Sight difficulties?	☐	
Learning difficulties?	☐	
SMI register?	☐	

Mental health issues that may impact on engagement?	☐																	
Mental capacity assessment required?	☐																	
Known safeguarding concerns?	☐																	
Mobility requirements (unable to climb on/off bed)?	☐																	
Mobility requirements	Walking	☐	Chair (Self Transfer)	☐	Chair	☐	Trolley	☐	AMB C1	☐	AMB C2	☐	AMB Car	☐	Own Chair	☐		

BACKGROUND INFORMATION / RISK FACTORS							
BMI *(Required for MRI limitations)*				Smoker/ex-smoker		’	
Alcohol				Other please specify			
Ethnicity							
Relevant family history							
Breast cancer	☐	Member		Age when diagnosed		*Details:*	
Prostate cancer	☐	Member					
To avoid undue delays to MRI for straight to test pathways				**Dates:**		**Details:**	
Does the Patient have a Pacemaker / other metal implants?	☐						
Has the patient had a joint replacement	☐						

PATIENT MEDICAL HISTORY		
Existing conditions	☐	
Current medication	Acutes	
	Repeats	
Allergies	☐	
Anticoagulants / Antiplatelets	☐	
Diabetic	☐	
Immunosuppressant	☐	

Additional information / Resources:

Referral Criteria for Prostate (NG12) - Overview | Suspected cancer: recognition and referral | Guidance | NICE
NICE Assess a person with suspected prostate cancer (June 2025) - Assessment | Diagnosis | Prostate cancer | CKS | NICE

Assessment | Diagnosis | Prostate cancer | CKS | NICE

Diagnostic value of the abnormal digital rectal examination in the modern MRI-based prostate cancer diagnostic pathway - PMC

Diagnostic uncertainty please use A&G / Advice & Refer pathway if locally available.

IF NHS E-REFERRAL IS UNAVAILABLE, COMPLETE FORM AND EMAIL TO THE REFERRAL TEAM WITHIN 24 HOURS.

To Note: These forms will be reviewed as part of regular cancer auditing